

1. Administrative & Study Identification

Full Study Title: A Phase Ib/II, Open-Label, Multicenter, Randomized Umbrella Study Evaluating the Efficacy and Safety of Multiple Treatment Combinations in Patients With Gastric or Gastroesophageal Junction Carcinoma (MORPHEUS C-Gastric and Gastroesophageal Junction Carcinoma) **Brief Title:** An Umbrella Study Evaluating the Efficacy and Safety of Multiple Treatment Combinations in Patients With Gastric or Gastroesophageal Junction Carcinoma **Short Title/Acronym:** MORPHEUS C-Gastric and Gastroesophageal Junction Carcinoma **Protocol Number:** Y043408 **NCT Number:** NCT05251948 **Posting ID:** 1150099463511524645 **Sponsor/Company Name:** Hoffmann-La Roche / Roche **Investigational Product:** Atezolizumab (Tecentriq) + Capecitabine + Oxaliplatin (Eloxatin, ATC Code: L01XA03) +/- Tiragolumab **Phase of Development:** PHASE1, PHASE2 (Phase Ib/II) **Trial Status:** COMPLETED (Study has been completed) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy (measured by Objective Response Rate [ORR]) and safety of multiple targeted treatment combinations in patients with Gastric or Gastroesophageal Junction Carcinoma. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DoR), and to characterize the safety and tolerability profile of the investigational combinations.

2.2 Background & Rationale To address the medical necessity of finding more effective first-line treatment combinations for patients with inoperable locally advanced, metastatic, or advanced Gastric or Gastroesophageal Junction Carcinoma (GC/GEJC). This umbrella study is designed with the flexibility to open new treatment arms as new treatments become available, close existing treatment arms that demonstrate minimal clinical activity or unacceptable toxicity, and rapidly identify synergistic combinations (such as anti-PD-L1 plus anti-TIGIT therapies) to improve patient outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Umbrella Study) **Control Type:** Active Comparator (Standard of care / Control Arm) **Blinding:** Open-label **Randomization:** Randomized

3.2 Planned Enrollment & Duration Targeted Enrollment (\$N\$): 40 patients **Number of Sites:** Multicenter **Estimated Study Start:** 01-Mar-2022 **Trial Status:** COMPLETED

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Inoperable locally advanced, metastatic, or advanced Gastric Carcinoma (GC) or Gastroesophageal Junction Carcinoma (GEJC). 2. Adenocarcinoma confirmed as the predominant histology. 3. Must not have received prior systemic therapy for advanced or metastatic disease.

4.2 Exclusion Criteria 1. Prior treatment with CD137 agonists or immune checkpoint blockade therapies. 2. Treatment with investigational therapy within 28 days prior to initiation of study treatment, or any contraindications to any of the study drugs of the chemotherapy regimen. 3. Patients with a signet ring cells dominant carcinoma, symptomatic/untreated central nervous system (CNS) metastases, or history of leptomeningeal disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Atezolizumab is administered by Intravenous (IV) infusion on Day 1 of each 21-day cycle, alongside chemotherapy (capecitabine and oxaliplatin [Trade Name: Eloxatin; ATC Code: L01XA03]) with or without Tiragolumab depending on the assigned treatment arm. **Route of Administration:** Intravenous (IV) and Oral. **Duration of Treatment:** Participants will receive treatment until unacceptable toxicity, progressive disease, or loss of clinical benefit as determined by the investigator.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications required to manage chemotherapy-induced side effects (e.g., antiemetics). **Prohibited:** Concurrent administration of other investigational agents, live vaccines, or prohibited immunosuppressive therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Baseline Scans, Labs, Biomarker Testing
Baseline	Day 0	Randomization, First Dose Administration	
Interim	Day 1 of 21-day cycles	Safety Labs, Efficacy Assessment (Tumor Scans every 6-9 weeks), Infusion	End of Study
At Progression	Final Vital Signs, Exit Interview, IP Accountability, Transition to Follow-up		

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Objective Response Rate (ORR) and incidence/severity of adverse events (Safety and Tolerability) assessed throughout the treatment duration.

Measurement Method: Investigator assessment using Response Evaluation Criteria in Solid Tumors (RECIST v1.1) and adverse event grading per CTCAE criteria.

7.2 Secondary & Exploratory Endpoints Progression-Free Survival (PFS) Overall Survival (OS) Duration of Response (DoR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via patient interviews and safety labs at each 21-day cycle visit. Specific focus on immune-related adverse events (irAEs). **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee (iDMC) regularly reviews safety data across all umbrella arms to recommend opening, modifying, or closing cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes. Safety Set includes all patients who received at least one dose of the study drug. **Sample Size Justification:** Target $N=40$ provides sufficient power to identify preliminary efficacy signals (ORR) in specific combination arms to justify phase 3 expansion. **Handling of Missing Data:** For ORR, patients lacking valid post-baseline assessments will be considered non-responders. Time-to-event endpoints (PFS, OS) will be censored at the date of last contact.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Standard onsite and remote monitoring to ensure protocol adherence. **Audit Trail:** Utilizing validated EDC systems with secure eCRF locking and formal data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: Y043408 **Registry Number:** NCT05251948 **Posting ID:** 1150099463511524645 **Sponsor/Lead Organization:** Roche (Hoffmann-La

Roche) **Study Title:** MORPHEUS C-Gastric and Gastroesophageal Junction Carcinoma **Official Regulatory Title:** A Phase Ib/II, Open-Label, Multicenter, Randomized Umbrella Study Evaluating the Efficacy and Safety of Multiple Treatment Combinations in Patients With Gastric or Gastroesophageal Junction Carcinoma (MORPHEUS C-Gastric and Gastroesophageal Junction Carcinoma)

12. Clinical Framework (Gastric Cancer Specific)

Disease Areas: Gastric and Gastroesophageal Junction Carcinoma

Primary Condition: Carcinoma (Preferred Name: Carcinoma)

Preferred UMLS Name: Gastric Carcinoma **Semantic Type:** Neoplastic

Process Condition Definition: A malignant tumor arising from epithelial cells. Carcinomas that arise from glandular epithelium are called adenocarcinomas, those that arise from squamous epithelium are called squamous cell carcinomas, and those that arise from transitional epithelium are called transitional cell carcinomas. **Clinical Codes:** MeSH: D002277 | HPO: HP:0030731 |

SNOMED: 1187425009 **Diagnosis Threshold:** Histologically confirmed, inoperable locally advanced, metastatic, or advanced adenocarcinoma

Medical Methodology: First-line systemic

immunochemotherapy **Optimization Target:** Maximal tumor response and durable disease control

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for

advanced GC/GEJC integrating chemotherapy and dual immune checkpoint blockade **Education Protocol (HCP):** Protocol training

on administration of multiple investigational products and management of immune-related toxicities. **Education Protocol**

(Patient): Comprehensive informed consent process detailing the risks and benefits of an umbrella trial methodology. **Quality**

Audit Mechanism: Routine source data verification, centralized imaging review, and real-time safety signal tracking.

14. Observation & Follow-Up Schedule

Total Duration: Continuous until disease progression or

unacceptable toxicity, followed by survival follow-up **Onsite**

Visit Cadence: Every 3 weeks (Day 1 of each 21-day cycle) **Remote**

Monitoring Frequency: Follow-up for survival status typically

every 12 weeks post-treatment discontinuation **Checklist Review**

Interval: Radiographic tumor assessments typically every 6 to 9 weeks

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					